

The Global Burden of Mental Illness

Prevalence, Impact, and the Treatment Gap

A data-driven overview of global mental health conditions, their disease burden, and the persistent gap between need and treatment

Source: Our World in Data / IHME Global Burden of Disease (2023)

**For Public Health Policy Analysts & Mental
Health Program Planners**

Hundreds of Millions Affected: 1 in 3 Women and 1 in 5 Men Face Major Depression

1 in 3

women will experience major depression in their lifetime

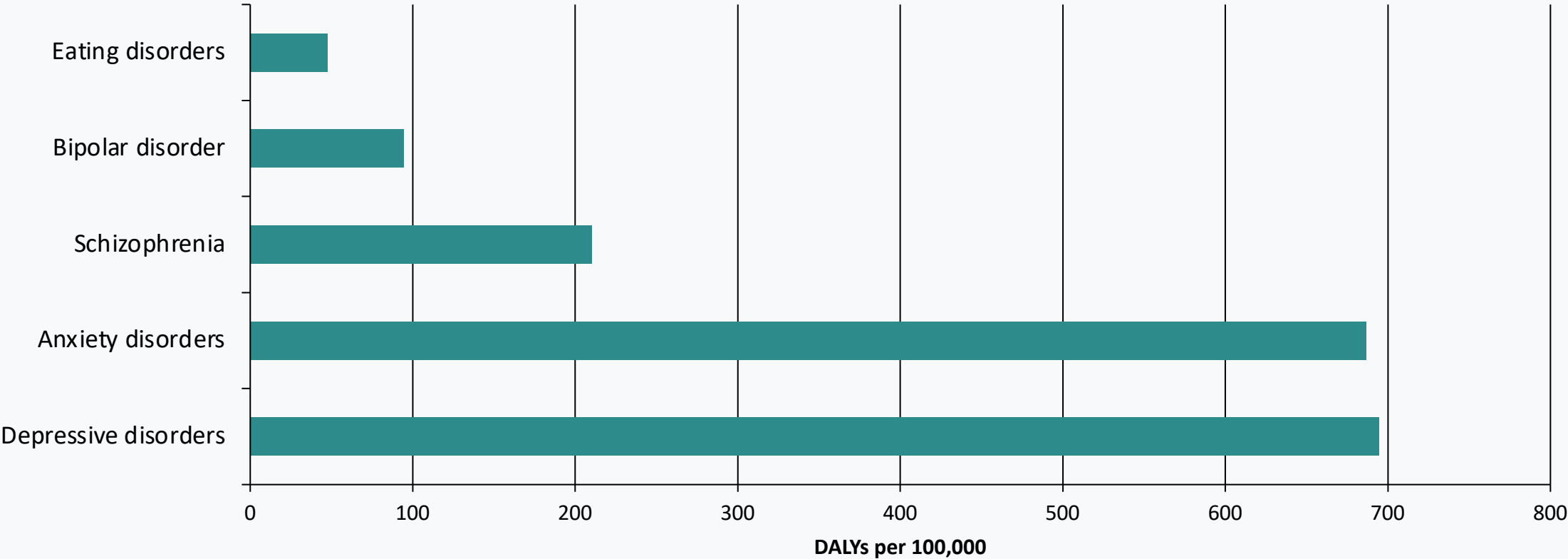
1 in 5

men will experience major depression in their lifetime

Hundreds of millions of people suffer from mental health conditions each year. Conditions range from common disorders — depression and anxiety — to less common but severe illnesses such as schizophrenia and bipolar disorder. Poor mental health affects well-being, the ability to work, and relationships with friends, family, and community.

Policy insight: The sheer scale of lifetime depression risk demands population-level prevention and early-intervention strategies.

Depression and Anxiety Dominate: ~695 and ~687 DALYs per 100K Respectively

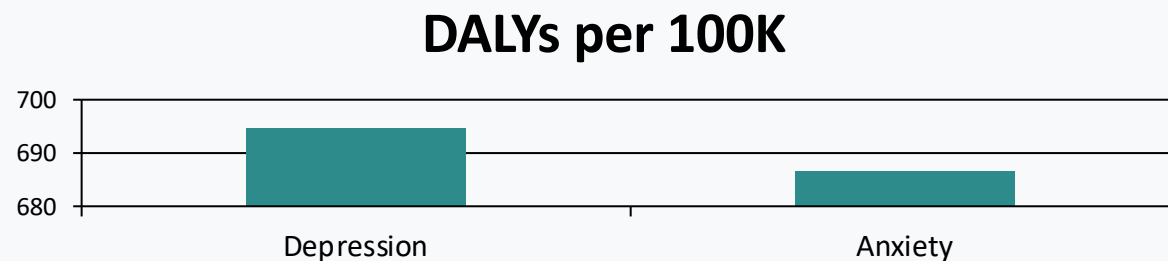


DALYs combine years of life lost due to premature death and years lived with disability. Depressive and anxiety disorders together account for roughly 80% of the total mental health disease burden across these five categories.

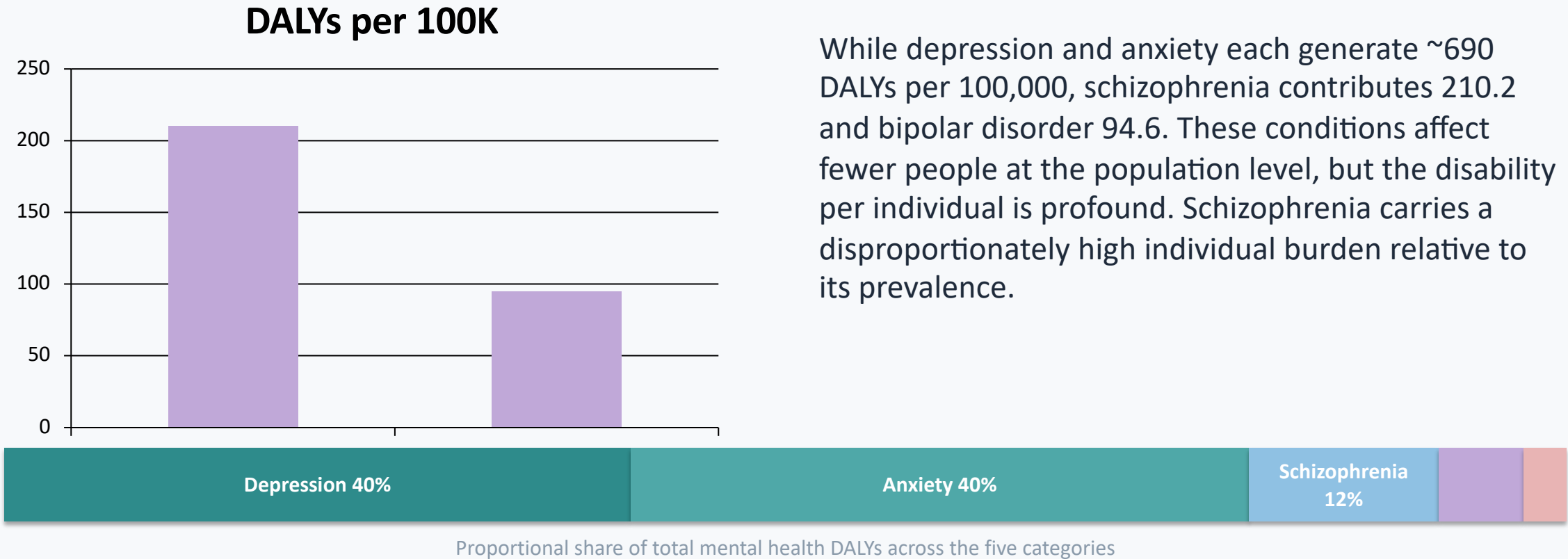
Depression and Anxiety Carry Nearly Equal Disease Burden Worldwide



Together, depressive and anxiety disorders dwarf the burden of schizophrenia, bipolar disorder, and eating disorders combined. This near-parity suggests policy and funding frameworks should treat depression and anxiety as co-equal priorities — not sequential ones.



Schizophrenia and Bipolar Disorder: Lower Prevalence but Severe Individual Impact



Treatment Is Available but Often Lacking or Poor Quality

Stigma

Access

Quality

Underreporting

Key implication: Mental illnesses are treatable and their impact can be reduced — yet significant barriers persist. Good data is essential to understand how, when, and why these conditions occur, and how they can be treated effectively and safely.

How People Cope: From Medication to Faith to Family Conversations

Prescribed Medication

**Religious or Spiritual
Activities**

**Talking to Friends or
Family**

The diversity of coping strategies underscores that mental health responses are shaped by cultural, economic, and healthcare system factors. Policy interventions must account for these differences rather than applying a one-size-fits-all approach.

Key Takeaways: Prioritizing Depression, Anxiety, and Closing the Treatment Gap

1. Depression and anxiety together represent the overwhelming majority of mental health DALYs — policy should treat them as co-equal priorities.
2. Schizophrenia and bipolar disorder carry disproportionate individual burden; specialized care must be maintained even as common-disorder services scale.
3. Effective treatments exist, but access and quality remain inadequate — especially in low- and middle-income countries.
4. Stigma suppresses both help-seeking and accurate data; reducing it is a prerequisite for closing the treatment gap.
5. Policy should prioritize scaling evidence-based interventions for common mental disorders while maintaining specialized care for severe conditions.

Evidence-based, culturally informed, and equitably distributed mental health care is both achievable and urgently needed.